Gainwell Technologies LLC Request for Information Response to

DEPARTMENT OF HEALTH AND HUMAN SERVICES

Centers for Medicare & Medicaid Services

Attention: CMS-0042-NC

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Request for Information

Health Technology Ecosystem

June 16, 2025

gainwell

Health Technology Ecosystem RFI

B. Patients and Caregivers

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve patients and caregivers. While we certainly want patients and caregivers to answer questions in this section (and in other sections) from the patient/caregiver point of view, we also invite all stakeholders to provide their viewpoints on the patient/caregiver workflows.

1. Patient Needs

PC-1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

An app that is a holistic health companion can proactively guide members and caregivers through their healthcare journey, anticipate needs, foster health literacy, and seamlessly connect members to personalized care and resources. This type of app is a proactive, hyper-personalized, and integrated digital health companion. It would be personalized so that it stays with the person, regardless of their insurance coverage changes (Medicare, Medicaid, Exchanges, or Commercial/other).

a. What are the top things you would like to be able to do for you or your loved ones' health that can be enabled by digital health products?

The top methods to assure a loved one's health are as follows:

Easy Access to Health Information

- **Medical Records:** View visit summaries, discharge instructions, active health issues, surgical history medications, allergies, and immunizations.
- **Test Results:** Receive timely updates when lab or imaging results are ready, often accompanied by physician comments.
- **Complete Health History:** Accessing all health records in one place helps patients to better understand their health and to make informed decisions. This includes a longitudinal health record that is mobile and includes information across all providers (not just those within a single system).
- **Access to living will and healthcare power of attorney documents**

Direct Communication with Benefits Administrator

- **Secure Messaging:** Send non-urgent questions or updates directly to your care team without needing to phone.
- **Benefits Guidance:** Receive advice and clarifications about benefits, limits, and other questions.

Empowerment Through Engagement

- **Active Participation:** Having access to your data encourages greater involvement in your care and adherence to treatment plans.

- **Patient, caregiver and interdisciplinary care team access** to Care Management plans and goals (when applicable).
- **Health Tracking:** Monitor trends such as blood pressure or glucose, set personal goals, and track progress.
- **Family Management:** With permission, manage healthcare for children or dependents from a single account.
- **Peace of Mind:** Real-time updates and messaging reduce the stress of waiting for critical information.

Convenient Appointment Management (including health related social needs)

- **Schedule Visits:** Book or request appointments online anytime.
- **View Upcoming Appointments:** Check date, time, location, and instructions.
- **eCheck-In:** Save time by completing forms, updating info, and paying co-pays before your visit.
- **Cancel or Reschedule:** Manage appointments with just a few clicks.
- **Telehealth Options:** Attend virtual visits from the comfort of your home.

Prescription and Medication Management

- **Refill Requests:** Quickly request prescription renewals.
- **Medication Review:** View a current list of prescribed medications and dosage instructions. Information on what medical conditions prescribed medications are treating.

Billing and Payments

- **View Bills:** Access itemized medical statements.
- **Online Payments:** Pay co-pays and bills securely.
- **Cost Estimates:** Receive price estimates for certain services, when available.

Universal Health Score

- Introduction of a single, comprehensive, dynamic measure of an individual's health status.

b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

A personal assistant could help with the following health needs:

- **Leverage Predictive Analytics of member data** (claims, clinical history, social determinants of health, and app usage patterns) to predict potential health risks, identify care gaps, and anticipate individual needs before they become urgent.
- **Personalized Health Nudges and Reminders:** Based on predictive insights, an app that can deliver highly personalized, timely nudges for preventive screenings, medication adherence, appointment reminders, and suggestions for healthier lifestyle choices (for example, annual check-ups or methods to manage blood sugar).

- **Behavioral Science Integration:** Messages and recommendations will be designed with behavioral science principles to motivate and encourage healthy actions, moving beyond simple reminders to foster lasting behavioral change.
- **Adaptive Communication:** An app that will learn and adapt to individual member preferences for communication channels (text, push notification, in-app message, and a prompt for a call from a care navigator) and timing, verifying messages are received and acted upon.

PC-2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

a. If so, what are some examples of benefits it has provided?

b. If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?

c. Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

The benefits of providing health information in one location include being able to view and share knowledge with caregivers and various types of providers. Some of the more valuable features include the ability to ask questions of providers, verify medications are refilled in a timely manner, and allows more timely follow-up with providers. An unstructured or lack of a standard data model makes it difficult to exchange information across platforms.

The Interoperability capabilities leveraging FHIR for Patient Access and allowing a member to access their data via a selected third-party application provides a simplified and efficient method for access. With the introduction of CMS-0057, providing access for providers extends this value to the point of care. Gainwell's Interoperability Solution under the Gainwell Connect umbrella delivers this service to more than 30 million Medicaid members across the country.

By combining this service with our GainwellGo (MDP) service, members have standardized access to their claims, encounters, medications, and other clinical data, as well as a modern application that provides a broader range of services.

The Universal Health Score serves as a simple indicator of patient health that encourages engagement and promotes self-management. The indicator can include social determinants and be visualized in culturally-sensitive and literacy-adapted formats. The score relies on interoperable APIs (FHIR and bulk FHIR) and broader USCDI+ elements to compute a complete score.

PC-3. Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?

There are multiple health management, care navigation, and personal health record apps that are useful in managing the different facets of the healthcare and wellness journey. For example, Gainwell's native mobile app, GainwellGo, has an extensive range of features that can be

leveraged by Medicare beneficiaries to navigate and provide a comprehensive member engagement journey.

GainwellGo differentiates itself from the market by:

An intuitive, mobile-first platform with secure access to health records

- Gainwell delivers a secure, mobile-first platform that provides intuitive access to health records, claims, and digital ID cards, enabling members to manage their healthcare needs seamlessly, anytime and anywhere.
- Through ongoing updates and enhancements, Gainwell verifies that the platform evolves to address the dynamic requirements of Medicaid programs and member needs.
- With streamlined and accessible designs, Gainwell prioritizes usability, simplifying complex processes to enhance member satisfaction and confidence in managing their benefits.

Multilingual support and inclusive design make services accessible to diverse Medicaid populations

- Gainwell's support for more than 20 languages and inclusive design principles provides accessibility and usability for diverse Medicaid populations, eliminating language and cultural barriers.
- Tools and platforms are designed to resonate with different communities, fostering equal access to essential healthcare services.
- Gainwell employs personalization to tailor experiences to individual member needs, enhancing engagement and fostering trust among underserved populations.

Streamlining communications through digital solutions

- Gainwell replaces expensive physical mailings with secure, cost-effective digital communication channels, significantly reducing operational overhead and enhancing efficiency.
- Digital correspondence provides faster, more reliable delivery of essential information, improving member satisfaction and reducing administrative delays.

PC-4. What features are missing from apps you use or that you are aware of today?

a. What apps should exist but do not yet? Why do you believe they do not exist yet?

An app that should exist is personalized and proactive to a member's chronic health condition and integrated with coverage information with a personalized Artificial Intelligence (AI) coach. For example, a beneficiary with a chronic metabolic condition such as diabetes. Another example is an aspirational app that tracks glucose and exercise metrics, but also integrates with a Part D Medicare plan showing which glucose monitors are covered and at what percentage, a member's out-of-pocket costs, and real time information on what is the remainder on their deductible.

Additional features include caregiver and family-shared access to member's health records, as well as tools integrating home care providers.

Apps with the features described above do not currently exist due to data silos, delays in widespread adoption, and standardized data sharing across providers, payers, and pharmacies.

Medicare (Parts A, B, C, D) and Medicaid business complexity, gap plans, regulatory burden, and a lack of clear business models are a hindrance to app developers from investing in this space.

b. What set of workflows do you believe CMS is uniquely positioned to offer?

CMS is positioned to offer a comprehensive, mobile first platform strategy that beneficiaries, payors, and service providers can leverage. This is an ecosystem where a member can leverage an app to act upon or make decisions on their care management, similar to what is currently provided under Patient Access APIs.

The vision extends to payor administrators who could leverage this same mobile platform for enhanced communication and administrative tasks. This includes:

- Direct communication with beneficiaries
- Sending broadcast messages
- Performing address and work verification tasks by engaging beneficiaries directly through the app

Further enhancements to the platform could include real-time, two-way communication between payor admins and beneficiaries, fostering immediate and efficient interactions.

Additionally, the platform could incorporate low-code/no-code options. This would enable payor administrators to easily create and send periodic surveys or data gathering forms to beneficiaries without requiring developer involvement, streamlining data collection and feedback processes.

PC-5. What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process?

What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

b. What changes would enable timely access to high quality CMS and provider generated data on patients?

To encourage patient and caregiver interest in digital health products, easy access is necessary, such as ID verification and the ability to share data across platforms. In addition, security, privacy, and safety of information is essential. By providing access to services and resources within the application environment, members are more likely to engage with the tools.

Today, AI is becoming an integral part of digital health solutions and will soon be used by all patients and caregivers. When it comes to technology, AI has the potential to address many challenges – from product design to reimbursement adjustments – making its applications limitless.

Because of this, CMS should focus on enhanced security, privacy, and safety for both AI systems and the data they handle. AI offers valuable, data-driven insights and predictive analytics that can save states thousands of dollars while increasing member engagement.

Embracing AI thoughtfully and securely will validate patients receive the most effective and trustworthy digital experiences.

PC-6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

To make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, it is important to have easy access (for example, ID verification) and the ability to share data across platforms. Security, privacy, and safety of information are also necessary features.

Additional important features include a simple and intuitive user interface, 508 accessibility, offline access or low-data usage mode, delegated/proxy access, and multi-device compatibility. The focus is on the features and services that a digital app can provide. The more value an app can deliver to individual user, the more likely they are to use it, and the more likely app developers are to engage in the space.

Our products are built with a deep understanding of the core needs to Medicare beneficiaries and caregivers. In addition to Medicare, it is important to note that Medicaid encompasses more than just healthcare, as it includes housing (home-based waivers), employment, and overall quality of life for all Medicaid populations.

Additionally, key use cases include families. Digital health products should focus beyond individual health to family health – whether it is a single mother and her child or a family caring for an adult with a disability. Accessibility is not merely a checkbox for compliance; it is baked into the entire product lifecycle. Through data-driven research, and continuous feedback from clients, digital health tools must be easy to use and meet the needs of Medicare beneficiaries.

PC-7. If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

To collect real-world data on digital health products' impact on health outcomes and related costs after they are released into the market, it is important to define hard and standard KPIs on key outcomes. This includes cost, quality, experience, and health outcomes.

With recent developments in digital therapies, providing a mechanism for entities creating these unique digital therapies to use the data for research and intelligence would drive advanced development.

2. Data Access and Integration

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?

c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

Health data that is readily available and valuable to patients or their caregivers includes claims, encounters, medical and pharmacy (linked), clinical (such as diagnosis), and immunization records, along with health-related social needs.

Challenges that app developers face are limited standardization and access to data sources such as social determinants, pricing, benefits under waivers, transportation, and food benefits.

Data silos currently exist in the health industry that CMS can help influence through legislation, workgroups, and the publication of industry best practices.

In addition, the early publication of proposed rules and use cases by healthcare regulatory bodies will further help app developers plan on standardization and interoperability.

Patients and caregivers often face challenges such as accessing quality providers and obtaining accurate information about Medicaid programs and resources. This could be reviewing data that reflects favorable opinions about a provider or using predictive insights to support healthier lifestyle choices and program knowledge. Additionally, they need to be able to connect directly with an individual for personalized, one-on-one assistance, receiving the support and information they need quickly, securely, and reliably. This is accomplished through adaptive communication, such as text, push notification, and in-app messages.

PC-9. Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

a. What difficulties are there in accessing or utilizing these data sources today?

b. What suggestions do you have to improve the Blue Button 2.0 API experience?

c. Is there non-CMS data that should be included in the API?

CMS data sources developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers include behavioral health and social determinants.

PC-10. How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?

a. Please provide specific examples.

b. What changes would you suggest?

c. What use cases could have a significant impact if implemented through TEFCA?

d. What standards are you aware of that are currently working well to advance access and existing exchange purposes?

e. What standards are you aware of that are not currently in wide use, but could improve data access and integration?

f. Are there redundant standards, protocols, or channels that should be consolidated?

g. Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

The key issues related to TEFCA are the disconnect between clinical data and claims-based data, as well as the timing of the data in terms of actionability. Most Medicaid programs struggle to capture clinical data as part of their overall member management programs. A key factor in this disconnect is the provision in HIPAA for “minimum necessary” that limits the amount of data shared with the payer and is focused only on the data needed to pay a claim. This limitation means that results and other clinical data is not available to make decisions on ongoing member care.

The issue associated with timing is the result of the delay in claims data that makes active member management difficult. TEFCA could be leveraged to share a broader range of data with the Medicaid programs that allows near real-time care management, improving the overall health of the Medicaid beneficiaries.

Ultimately, the goal is to align HIPPA with modern patient needs related to a more integrated and wholistic view of their care and data across provider and payer entities.

PC-11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

a. How valuable, available, and accurate do you find the data they share to be?

b. What changes would you suggest?

c. Are there particular examples of high-performing HIE models that you believe should be propagated across markets?

d. What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

HIEs are currently advancing patient access to health information in the real world by standardizing and aggregating data across payers, providers, and other entities. The use of HIE master patient indexes (MPIs) is helpful in matching clinical and claims data within the HIE. However, there is still a challenge translating the clinical data into systems that can leverage the data. Medicaid systems struggle with the integration of clinical data, including MMIS and EDW platforms that have not traditionally integrated this data type.

PC-12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

a. Examples may include the following:

(1) Binding cost estimates for pre-defined periods.

(2) Viewing provider schedule availability.

(3) Using third-party apps for appointment management.

(4) Accessing patient-facing quality metrics.

(5) Finding the right provider for specific healthcare needs.

b. What use cases are possible today?

c. What should be possible in the near future?

d. What would be very valuable but may be very hard to achieve?

The most valuable operational health data use cases for patients and caregivers that create more efficient care navigation or eliminate barriers to competition among providers is as follows:

- Standard provider performance metrics
- Network coverage information (personalized)
- End-to-end journey information

3. Information Blocking and Digital Identity

PC-13. How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

CMS can encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal by showing the value they receive by sharing information in tangible terms.

PC-14. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP)):

a. What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?

b. What could be the benefits to patients/caregivers if digital identity credentials were more widely used?

c. What are the potential downsides?

d. How would encouraging the use of CSPs improve access to health information?

e. What role should CMS/payers, providers, and app developers have in driving adoption?

f. How can CMS encourage patients to get digital identity credentials?

Current challenges include the lack of local usability of federal credentials.

C. Providers

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve providers. While we certainly want providers to answer questions in this section (and in other sections) from the provider point of view, we also invite all stakeholders to provide their viewpoints on the provider workflows as appropriate.

1. Digital Health Apps

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

a. What are the current obstacles?

b. What information should providers share with patients when using digital products in the provision of their care?

c. What responsibilities do providers have when recommending use of a digital product by a patient?

To receive better care, CMS and its partners can encourage providers to leverage digital health products for their patients by offering provider access to patient information via an app. In addition, reimbursement for time spent is an incentive.

Current obstacles include:

- Low patient adoption and usage rates of digital health products, particularly among elderly and low-income populations.
- Siloed information linked to single-provider system EHRs limits usefulness for patients that access multiple systems for their care. Information is fragmented, incomplete, and less useful to both providers and patients/caregivers.

Providers should fully disclose their use of available digital products with their patients. Shared information should include access to the full medical record with special attention to summaries of active medical problems, medication lists, lab tests, treatment plans, upcoming appointments, and electronic messaging.

In addition, providers bear the responsibility to assure that recommended digital products are fully vetted for critical security measures, interoperability, and appropriate user interfaces.

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Obstacles could be mitigated through improved training, including peer-based sessions. Other methods include showing and conveying tangible benefits. In regard to impacting productivity numbers, accessing an automation tool should not result in providers being required to see more patients. Effective utilization is also inhibited by the lack of one system for all payer reporting.

Obstacles include the following:

- Accessing applications is often outside normal physician EHR and clinical workflows, requiring additional time and effort.

- Lack of incentives to engage, including financial or perceptions of easing physician workloads, or improving quality of care.

Obstacle mitigation includes the following:

- Providing incentives to demonstrate that using applications at the point of care results in increased physician productivity, decreased physician workload, and how applications can improve quality or care for patients.

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)?

Please address all of the following:

a. Current challenges in accessing different data formats.

b. Impact on patient care quality.

c. Technical barriers to full data accessibility.

d. Cost or privacy implications of making all data formats interoperable.

e. Priority level compared to other interoperability needs.

Our experience has proven that unstructured and unstandardized data adds a burden to everyone in the ecosystem.

Current challenges are as follows:

- Lack of interoperability and data consolidation results in fragmented medical record information that is based on healthcare systems and providers accessed, rather than patient based.
- Lack of longitudinal patient-based medical records results in decreased quality of care as well as significant waste in duplicated diagnostic testing and treatments.

Structured data is obviously the most easy to share across platforms. However, access to all medical record information, including unstructured data, is still desirable if possible.

PR-4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

Changes or improvements to standards or policies include the standardization of data and APIs.

2. Data Exchange

PR-5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

a. Patient Access API

Gainwell currently provides the Patient Access API for 22 states under the CMS-9115 Final Rule. The Patient Access API can be accessed via a patient-selected third-party app with individual queries based the individual patient authentication token.

b. Standardized API for Patient and Population Services

Gainwell does not support or use Standardized API for Patient and Population Services in its provider organization's systems.

c. Provider Directory API

Gainwell currently provides the Provider Directory API for 22 states under the CMS-9115 Final Rule. Provider Directory API is a public API that performs bulk transactions for general access/lists and individual transaction models for patient look-up/mapping from claims/encounters.

d. Provider Access API

Gainwell is deploying a CMS-0057 Final Rule solution for its current 9115 clients with provider access as a part of the solution. The Provider Access API is a bulk transaction process that leverages the providers EHR as the accessing application and downloads a list of attributed members with the proper consent. After the provider selects a member from the download, an individual transaction is leveraged to draw down the member data.

e. Payer-to-Payer API

Gainwell is deploying a CMS-0057 Final Rule solution for its current 9115 clients with payer-to-payer access as a part of the solution. The Payer-to-Payer API leverages a bulk transaction that moves data from one payer interop platform to another based on proper member consent. It is based on an individual member record, but transfers data (excluding financial data) from one payer to another. The data is then ingested into the interop platform of the receiving payer.

f. Prior Authorization API

Gainwell is deploying a CMS-0057 Final Rule solution for its current 9115 clients with Prior Auth API (ePA) as a part of the solution. The ePA uses the provider's EHR as the end user application receiving data (CRD and DTR) from the servicing payer entity to the provider's EHR. It then sends the complete PA with clinical data attached to the receiving payer's UM platform.

g. Bulk FHIR – Do you support Group ID-based access filtering for population-specific queries?

Gainwell currently has one customer doing population-specific bulk FHIR transactions to download data into a third-party system that processes the data for expanded analytics and services.

h. SMART on FHIR – Do you support both EHR-launched and standalone app access? What does the process for application deployment entail?

SMART on FHIR is the primary service deployed with all APIs. The Developer Portal supports access for third-party apps as well as EHR vendors to apply and be granted to access to state end points. Each state has the ability to manage application access for their given environment.

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

CMS can implement prior authorization-related data. This encompasses the first submission as well as medical-necessity evidence that is standardized and easily shared across entities.

Typically, aggregated patient clinical data from the HIE is a preferred method of accessing data, at least from the Medicaid perspective. If the HIE is standardizing data from their various clinical providers and accepting an eligibility file from the Medicaid program, this data can be easily extracted and included in the deployed interoperability solutions. There is tremendous value in providing near real-time clinical data to providers from the payer, as the payer tends to have a more wholistic view of patient encounters and can reduce the impact of poor HIE participation.

The challenge, as described above, is the lack of clinical data shared with the payer and the difficulty in aggregating and collecting that data in an efficient manner. The interoperability solutions support the collection and matching of clinical data with claims and other payer data via the FHIR standard, but many HIEs do not normalize the data from their participants. This forces interop solutions to map each transaction individually. We suggest the ONC include data standardization in the EHR certification process to include FHIR clinical resources as the primary method of normalization across provider data sets.

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

a. What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?

b. In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?

c. Are there requirements CMS should consider for data registries to support digital quality measurement in a more efficient manner? Are there requirements CMS should consider for data registries that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

The bulk of the provider-related workflows and data needs are standard across the nation, but each state has different nuances. This should and could be standardized across the country via CMS to eliminate duplication of work and unnecessary overhead (even across Medicare and Medicaid).

3. Digital Identity

PR-9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?

b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?

CMS can encourage providers to accept digital identity credentials through ID verification via federal systems. This could significantly improve the patient experience. Similarly, provider-related workflows (for example, ID, enrollment in Medicaid, and credentialing) could be standardized and managed centrally.

Most state Medicaid programs do not leverage an identity broker such as ID.me for member level access to systems. A limited number of member access capabilities are currently being used. There are specific examples of state-level identity management tools being used but, in many cases, those state-level tools do not include the Medicaid ID as an element in the tool. Consequently, Medicaid programs do not find value in using the tools. This results in each member-facing application to develop and maintain its own identity management tool and isolated, be-spoke solutions that cannot be leveraged elsewhere. This creates a less seamless experience for members across various programs.

PR-10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs):

a. What are the challenges and benefits for providers?

b. How would requiring their use improve access to health information?

c. What are the potential downsides?

d. What impact would mandatory credentials have on a nationwide provider directory?

e. How could digital identity implementation improve provider data flow?

f. Would combining FHIR addresses and identity improve data flow?

Nationwide provider credentialing and ID management would assist in solving many pain points for a variety of stakeholders.

PR-11. How could members of trust communities3 (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?

While TEFCA is designed to support payers and payer data, it has limited use in this space. The issue is not necessarily the security standards (IAL2, for example), it is the ability for a payer to

access clinical data for a member within the scope of HIPAA's "minimum necessary" limitation of clinical data sharing. An optimal situation is the re-alignment of HIPAA to remove minimum necessary and coordinate the ONC's EHR standards specifically related to patient portals with the CMS rules related to interoperability. This creates a single, cohesive patient/member experience platform. In addition, it combines all health data in one place for the patient, with the ability to decide who should have access to their data (consent).

4. Information Blocking

PR-12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

Yes, ASTP/ONC should consider removing or revising information-blocking exceptions or conditions within the exceptions.

PR-13. For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

Data blocking is less about the individual provider, and more about the EHR vendor and their disincentives to sharing data outside of their solution. Data blocking penalties are valuable and would be more important if there was actual enforcement, resulting in real penalties to EHR vendors that violate the provision. An EHR vendor sharing data within its own ecosystem should not be considered data sharing within the law if they block sharing outside of that ecosystem or make access outside of their ecosystem difficult and costly.

D. Payers

PA-1. What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder participation in TEFCA?

HIPAA alignment to limit or eliminate the "minimum necessary" would reduce the restrictions related to clinical data sharing with payers.

PA-2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

By setting incentives for early adoption, CMS can encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers. Expanding the services supported by the CMS rules-based APIs such as expanded eligibility, member enrollment, provider enrollment, and other member services drives adoption and improves efficiency in payer programs. An example of this is the Prior Authorization API in CMS-0057 that provides real value in fixing the prior authorization issues that exist today.

PA-3. How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?

CMS can encourage payers to accept digital identity credentials by setting incentives or penalties. In addition, CMS could ask states that have identify management tools to include Medicaid membership as a part of their tool set to leverage what is already in place.

PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

This would be a game changer for the industry. Keeping provider information up to date is currently a significant pain point and costs the ecosystem a great deal of money.

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

a. How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?

b. In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

These technology advances would tremendously simplify prior authorization processes.

PA-7. How can CMS encourage payers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

CMS could make reporting information blocking the condition of participation. The impact would be improved compliance and transparency.

E. Technology Vendors, Data Providers, and Networks

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve technology vendors, data providers, and networks. While we certainly want technology vendors, data providers, and networks to answer questions in this section (and in other sections) from their point of view, we also invite all stakeholders to provide their viewpoints on the technology vendor, data provider, and network use cases as appropriate.

1. Ecosystem

TD-1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Short- and long-term steps include financial incentives and access to data.

One of the interesting trends Gainwell has observed is through our GainwellGo Mobile Digital Platform (MDP). For states that have deployed MDP and are using it for access to the Interop Patient Access API, the number of member users is in the thousands. For states without MDP, the number of users is less than 100. By providing an app that offers other services, Digital ID cards, and messaging, the uptake in using the APIs dramatically increases.

Other steps include granting caregiver access to records and tools in a timely manner. Developer challenges can be addressed. Sandbox environments can be provided with de-identified data. Streamlined regulatory pathways encourage digital product development.

TD-2. Regarding CMS Data, to stimulate developer interest--

a. What additional data would be most valuable if made available through CMS APIs?

b. What data sources are most valuable alongside the data available through the Blue Button 2.0 API?

c. What obstacles prevent accessing these data sources today?

d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

The following will stimulate developer interest:

- Claims, encounters, medical and pharmacy (linked), clinical (diagnosis), behavioral health, and social determinants.
- Common ID or blockchain-based decentralized digital identify (DID) systems offer secure, portable identity verification that providers and members can control. This improves cybersecurity and consent while meeting NIST standards.
- Security, privacy, and safety.
- Clarity on who owns the data.

As a rule, third-party apps derive revenue by providing other paid services on the app or from advertising revenue. If CMS could provide additional demographic information on the member (with member consent), this would allow for targeted advertising and draw more developers to build these apps. By adding value-based services for members (coupons, for example), more member use would drive additional developer engagement. We have witnessed this MDP deployment and other services that members receive from the app.

2. Digital Identity

TD-3. Regarding digital identity implementation:

a. What are the challenges and benefits?

b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?

c. What impact would mandatory use of the OpenID Connect identity protocol have?

Digital identity implementation would have tremendous benefits regarding simplification and standardization. The ability to establish a single identity service for members across all services would improve the beneficiary experience and reduce costs by eliminating the one-off identity tools being built.

3. Technical Standards and Certification

TD-4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

CMS can encourage using open, standards-based, and publicly available APIs over proprietary APIs through incentives and penalties. While enforcement of certain standards is a last resort, it carries a lot of weight through Blue Button and FHIR.

Optionally, CMS can mandate use of open APIs in CEHRT and reduce cost differential between proprietary and public standards.

TD-5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost?

A FHIR endpoint directory improves look-up, routing, and reduces integration costs. CMS or a national steward can take the lead in managing the public directory. Blockchain could be the shared infrastructure for publishing and querying a nationwide provider directory of FHIR endpoints.

TD-6. What unique interoperability functions does TEFCA perform?

a. What existing alternatives should be considered?

b. Are there redundant standards, protocols or channels or both that should be consolidated?

TEFCA represents a “network of networks.” By providing regional hubs that allow data providers to connect and share data between the hubs and their participants, TEFCA represents an “internet-like” infrastructure. The challenge with TEFCA is not the design or feature set; it is limited by actual data from provider participation. For example, if a given region has limited hospital or ambulatory provider participation, the amount of data on a patient is reduced and the value is diminished. Driving increased clinical and payer participation improves the ecosystem without changing the needed infrastructure support.

TD-7. To what degree has USCDI improved interoperability and exchange and what are its limitations?

a. Does it contain the full extent of data elements you need?

b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?

c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?

d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

The updates to USCDI have addressed specific data element limitations from previous versions. At this time, we do not see any issues with the current version, at least in the Medicaid space. As more Medicaid agencies engage in ingesting clinical data, this opinion may change. CMS can consider supporting flexible formats such as JSON, which is AI-parsable.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

The new HTI-1 and HTI-2 ONC rules are a game changer for leveraging the CMS Interop capabilities currently being deployed. Provider access and ePA APIs are meaningless without EHR access, and these ONC rules are driving that access. While meaningful use drove the use of electronic records, future certification requirements drive greater interoperability and accelerate data sharing that ultimately improves health and reduces costs.

TD-10. For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

Based on current CEHR standards, the following capabilities would improve the engagement between the payer and provider leveraging the technology:

- FHIR support of online and real-time eligibility checks
- Broader FHIR integration for patient portals, allowing a portal to access the payer's FHIR endpoints. This allows claims data to be consolidated into the current clinical data for a patient.
- FHIR-enabled admission and discharge notifications to the patient's payer and, in the case of transitions of care from inpatient to ambulatory, FHIR-enabled notifications to support that transition.

TD-11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). Such health IT developers are also required to publicly describe the format of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:

a. Should this capability be revised to specify standardized API requirements for EHI export?

b. Are there specific workflow aspects that could be improved?

c. Should CMS consider policy changes to support this capability's use?

Yes, these requirements should support FHIR-based APIs and should include the payer as a part of the receipt of the data. Given the real-time nature of this activity, payer-based care managers would have greater insight into the current status of their members and could provide a means through which individual members can be better supported. This entails adding the payer into the care team.

4. Data Exchange

TD-12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

CMS could consider endorsing based on transparency, accuracy, and standards adherence.

TD-13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

a. What are the primary obstacles to this?

b. What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

New opportunities and advancements include the ability to view all aspects of a patient's health journey, as well as connecting the dots across the ecosystem to identify improvement opportunities.

A plethora of data is available from external sources that improves the overall efficiency of the payer environment. For example, leveraging credit bureau data (Experian, Equifax, and Trans Union) provides insight into social determinant data that could improve outreach and ancillary services to beneficiaries to enhance overall health. In addition, using this data allows a payer to target populations for special programs. For example, using air quality data to inform a program focused on childhood asthma.

TD-15. Regarding bulk FHIR APIs:

a. How would increased use of bulk FHIR improve use cases and data flow?

b. What are the potential disadvantages of their use?

Bulk endpoints in FHIR provide an exceptional tool for access to populations or the bulk export of member data to use in analytics and other programs. An example is the bulk export of member data for geriatric program enrollment. The downside is the security concerns associated with bad actors that could potentially hack the endpoints to gain access to a significant number of members or a large amount of data on an individual member. In one case, a state Medicaid program chose to disable the bulk data endpoints to reduce the potential risk.

TD-16. What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

a. Do current rules encourage scalable network participation?

b. What changes would improve alignment (for example, API unification, reciprocal access)?

Shared infrastructure is more scalable. Reciprocal APIs and participation in common networks will improve alignment. A shared ledger could record access to patient data across networks, bulk FHIR transaction tracking, and reciprocal access enforcement.

TD-17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

To validate the viability of healthcare data sharing networks, CMS and ASTP/ONC should be appointed to administrative roles.

5. Compliance

TD-19. Regarding price transparency implementation:

a. What are current shortcomings in content, format, delivery, and timeliness?

b. Which workflows would benefit most from functional price transparency?

c. What improvements would be most valuable for patients, providers, or payers, including CMS?

d. What would further motivate solution development?

Price transparency is often delayed, lacks granularity, and is difficult to navigate. CMS could require APIs for machine-readable files and mandate structured pricing formats.

F. Value-Based Care Organizations

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve value-based care organizations. While we certainly want value-based care organizations to answer questions in this section (and in other sections) from the value-based care provider point of view, we also invite all stakeholders to provide their viewpoints on the value-based care workflows as appropriate.

1. Digital Health Adoption

VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Incentives include the ability to directly impact (and prove reduction in) healthcare costs and adoption. Bonus points could be offered in MSSP for digital tool use to accelerate adoption. Performance benchmarks can use measured improvements in universal health scores across the population.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

These key themes and technologies should be tied to healthcare outcomes and incentivized usage.

VB-3. What are essential health IT capabilities for value-based care arrangements?

a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.

b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

Essential health IT capabilities for value-based care arrangements include being able to view patient data end-to-end (health, non-health, behavioral health, medical/pharmacy), predictive analytics, multidisciplinary care teams, behavior change tools, and robust performance management.

VB-4. What are the essential data types needed for successful participation in valuebased care arrangements?

The essential data types necessary for successful participation in value-based care arrangements consist of health, non-health, behavioral health, medical/pharmacy, and social types.

3. Technical Standards

VB-11. What specific interoperability challenges have you encountered in implementing value-based care programs?

Interoperability gaps include mismatched patient IDs, non-standard APIs, and lack of event notifications.

VB-12. What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

Technology standardizations could allow value-added extensions while maintaining minimal core standards.

VB-13. What improvements to existing criteria and standards would better support value-based care capabilities while reducing provider burden?

Rather than reviewing dozens of metrics, a provider can review and act upon a single, dynamic universal heath score dashboard.

VB-14. How could implementing digital identity credentials improve value-based care delivery and outcomes?

Implementing digital identity credentials would improve attribution, consent management, and care continuity.

VB-15. How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

The FHIR directory would streamline look-up and data access. Key data elements to consider are endpoint type, NPI, specialty, contact, and geography. Universal heath score data can flow from multiple FHIR endpoints. Its calculation depends on robust, discoverable connections.